

1<sup>st</sup> week Wednesday case scenario

1-dr.

2-dr.

3-dr.

4-dr.

## Case 29

Mrs. Zainab Jabir visited you at the outpatient clinic. You are discussing her problem in the following dialogue:

- Hello doctor.
- Hello Zainab, how are you. I am ready to hear from you, what is your problem?
- I am 33-years-old, P2A2, married for 6 years.

G1: NVD of a live male baby, now he is 4.5 years old with normal milestones.

G2: my baby died inside my uterus at 7 months of my pregnancy and I delivered vaginally.

Then I diagnosed to have type 2 diabetes and I am now on:

Glibenclamide tablet 1x1

Metformin 500 mg 1x3

G3 diagnosed to have anencephalic fetus at 16 weeks and I got termination of my pregnancy.

G4 ended with missed abortion at 3 missing periods.

I consult my family doctor, he told me about the effect of badly controlled diabetes on a pregnant woman and her baby, he told me that I have to have very good control of diabetes in the pre-conception period to ensure better outcome.

So I am her to get your help for pre-conception counseling

☐ Yes by now you know that diabetes can affect many of your organs and we have to ensure that you are in a good health before allowing you to become pregnant.

We have to consult an ophthalmologist for -----  
----- Then to consult a  
nephrologist to -----  
-----

And a neurologist for -----

And a cardiologist for -----

Then we have to start good control of your blood glucose level. Good glycemic control in the pre-conception period would minimize the risks of-----  
-----and good glycemic control throughout pregnancy would prevent -----  
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- How long the pre-conception period is?

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.....  
.....  
.....  
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.....

- How can I achieve a good control of my blood sugar?

☐ I have to shift you from oral hypoglycemic agents to insulin and only allowing you to become pregnant when your HbA1c is-----

- How can I take insulin?

☐ There are about 3 insulin regimens:

1. -----  
-----  
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2. -----  
-----

3. -----  
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- What about the dose?

☐ The starting dose is 1 IU/Kg/day in divided doses and then to increase or decrease according to your sugar profile.

Our aim is to have pre-prandial blood sugar of -----

And post-prandial blood sugar of -----

Let us measure your body weight. Ok, it is 75 Kg.

Then we can calculate the dose of insulin according to the second regimen.

Morning dose -----

Evening dose-----

- How frequent should I measure my blood sugar

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- Shall I have to go to the hospital every time I have to measure my blood sugar?

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.....  
( a glucometer is supplied to the group & one of the student is going to be a volunteer for measurement of blood sugar )

- But this glucometer measure in mg/dl not in mmol/l  
☐ OK to change from mg/dl to mmol /l-----  
and to change from mmol/l to mg/dl -----

- How many types of insulin are available?

- ☐ 1-----  
2-----  
3-----  
4-----

(a leaflet with the container of each one of the types supplied to each group )

- Is it an IM injection?

- ☐ No, it can be used -----  
or ----- in cases of ketoacidosis.

- How can I measure the units of insulin?

( to supply each group with an insulin syringe )

- Shall I continue with the same dose throughout pregnancy?

☐ No, during the first trimester-----due to-----  
----- then during the 2nd and 3rd trimester the doses -----  
----- according to your sugar profile due to-----  
-----.

- Shall I deliver by C/S because of my diabetes?

☐ -----  
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